

CENTRAL CALIFORNIA
EMERGENCY MEDICAL SERVICES
A Division of the Fresno County Department of Public Health

Manual	Emergency Medical Services Administrative Policies and Procedures	Policy Number 510.32
Subject	Basic Life Support (BLS) Protocols ALLERGIC REACTIONS AND ANAPHYLACTIC SHOCK	Page 1 of 2
References	Title 22, Division 9, Chapter 3.1 of the California Code of Regulations	Effective Fresno County: 01/15/82 Kings County: 04/10/89 Madera County: 06/15/85 Tulare County: 04/19/05

STANDING ORDERS	
Assessment	ABCs
Secure Airway	Protect with position, basic airway maneuvers, pharyngeal airway as needed. Assist respirations as needed. Suction as needed.
Oxygen	No oxygen for mild reaction. High flow O ₂ if severe reaction/anaphylaxis.
Remove Allergen	If appropriate (i.e. bee stinger), apply cold compress.
Severe Reaction/Anaphylaxis	
Epinephrine	ONLY for anaphylaxis or severe allergic reaction with any of the following signs or symptoms: Severe respiratory distress, systolic BP < 90 mmHg, bronchospasm/wheezing, airway edema, stridor - Assist patient with taking their own prescribed epinephrine autoinjector, if available. Age ≥ 15 years: - Administer 0.3 mg Epinephrine (concentration 1mg/1ml) via autoinjector. May repeat once in 5 minutes if severe symptoms persist. (Authorized EMT/First Responder Only). OR - Administer 0.4 mg (0.4 ml) Epinephrine (concentration 1mg/1ml) intramuscular. May repeat once in 5 minutes if severe symptoms persist. (Authorized Optional Scope EMT only).
Transport	Per EMS Policy #547. Begin transport as soon as possible after initial stabilizing measures for severe allergic reaction. Rendezvous with ALS as soon as possible.

Approved By	Daniel J. Lynch	Revision
EMS Director	(Signature on File at EMS Agency)	07/01/2026
EMS Medical Director	Miranda Lewis, MD (Signature on File at EMS Agency)	

Subject	Paramedic Treatment Protocols - Allergic Reactions and Anaphylactic Shock	Policy Number 530.32
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SPECIAL CONSIDERATION AND PRIORITIES

1. Anaphylaxis is a severe allergic reaction characterized by any of the following:
 - Involvement of the skin/mucosa with respiratory compromise or hypotension
 - Hypotension after exposure to a known allergen
 - Involvement of two or more systems (skin/mucosa, respiratory compromise, gastrointestinal symptoms, hypotension)
2. Treat as an allergic reaction only if history of exposure to allergen (such as bee sting) or other signs of acute allergy – such as hives, itching, erythema, edema, stridor, respiratory distress, wheezing, or hypotension.
3. Initiate transport as soon as possible after initial dose of IM epinephrine for patients with severe allergic reaction. Transport lights/siren all patients in shock or unresponsive to therapy.
4. Be cautious in using Epinephrine in patients over 70 years of age, or patients with history of angina or hypertension. Watch BP, pulse, and monitor closely. Be extremely cautious with dosage calculations and administration.
5. Documentation must be completed in the electronic patient care record and include the following when epinephrine is administered:
 - Name of the provider performing the procedure
 - Drug name, concentration, dose, and administration site
 - Time of administration
 - Reassessment and complications, if any